

HEALTH SERVICES APPEAL AND REVIEW BOARD

Soraya Farha, Vice-Chair, Presiding
Taivi Lobu, Vice-Chair
Elizabeth Mullan, Board Member

Heard April 19, May 26 and October 13, 2010 at Toronto, Ontario

IN THE MATTER OF AN APPEAL UNDER SECTION 20(1) of the *Health Insurance Act*, Revised Statutes of Ontario, 1990, Chapter H.6, as amended

B E T W E E N:

C.T.C.T.Appellant

and

**THE GENERAL MANAGER,
THE ONTARIO HEALTH INSURANCE PLAN**

Respondent

For the Appellant: Annette Jansen, Counsel
 Jan Fisher, Student-at-law, April 19, 2010

For the Respondent, John Johnston, Counsel
General Manager, OHIP:

DECISION AND REASONS

I. DECISION

1. On January 5, 2009, Dr. Stephen Cudmore submitted a Prior Approval Application for Full Payment of Insured Out-of-Country Health Services to the Ministry of Health and Long-Term Care (the Ministry) with respect to health care services for the Appellant, Mr. C.T.. The application was for 60 – 90 days of inpatient and lab services for acquired brain injury and cocaine addiction at The Canyon, a treatment facility in the United States. The Ministry refused the application. The Appellant appealed the refusal to the Health Services Appeal and Review Board.

2. For the reasons set forth herein, the appeal is denied.

II. BACKGROUND

3. The Appellant, at 22 years of age, was involved in a very serious motor vehicle accident in 2004 in British Columbia, resulting in Traumatic Brain Injury, (TBI). The Appellant underwent six weeks of hospitalization in British Columbia, and then twelve months of treatment at Laurentian Hospital in Ontario, six months as an inpatient, followed by six months as an outpatient.
4. Subsequent to the Appellant's hospitalization, the Appellant's parents made private therapy arrangements for the Appellant (with for example, a speech therapist, physiotherapist, and an intervention therapist).
5. The Appellant's parents purchased an apartment building in trust for him, and exercised continuing powers of attorney, over his financial and medical affairs. The Appellant received an allowance through his parents. In these circumstances, the Appellant lived on his own. He was involved in a relationship, and in May 2007, became a father, though maintaining separate living quarters.
6. In the years following the accident, the Appellant developed serious drug and alcohol addictions. His family began researching options for treatment.
7. In the spring of 2008, the Appellant's parents contacted Homewood Health Centre. They spoke by telephone with a person on staff at Homewood Health Centre (Homewood) who advised that the facility would need to speak with the Appellant directly. The Appellant's family then advised that their son had an acquired brain injury and they wished to have a place that was ready to take the Appellant once he accepted that he needed help. Based on the information given to her by the Appellant's parents about the acquired brain injury, the person at

Homewood indicated to the parents that she did not think that he would be able to follow Homewood's program and group therapy.

8. In May 2008, the Appellant's parents contacted Homewood a second time but did not mention the acquired brain injury. Homewood stated that they needed a referral from his family doctor and that they would need to speak with him directly. The Appellant's family doctor, Dr. Cudmore, made an application Homewood in Guelph, Ontario.
9. Homewood sent Dr. Cudmore a letter dated July 24, 2008, indicating that the Appellant's name had been removed from its waiting list "because we did not receive requested information from the patient necessary to offer admission."
10. The Appellant's father testified that when he contacted Homewood to enquire about the application, he was advised that the Appellant had not returned the information forms that Homewood had mailed to his address. The Appellant's father asked if Homewood would send these forms directly to him and his wife, but was told that was not possible because of new privacy legislation relating to health information. He testified that he told Homewood that he had a Power of Attorney for his son, both financial and medical, but was informed that this made no difference and his offer to send Homewood the Power of Attorney information was declined. This concluded the parents' efforts to seek treatment for the Appellant at Homewood.
11. The Appellant's substance abuse continued. In November 2008, he threatened to kill both his father and then himself. He was committed into a hospital for three days, after which time he signed himself out of the facility.
12. On approximately December 23, 2008, at a party, the Appellant was hit in the head with a baseball bat, requiring stitches. Up until this point, according to the Appellant's father, the Appellant was not prepared to embark on rehabilitative

- treatment. On or about December 27, the Appellant decided that he wanted help. The family was of the view that the Appellant was in a serious crisis situation.
13. On December 29, 2008, the Appellant's mother made a number of phone calls to determine addiction treatment options in Ontario, to a number of facilities, but not Homewood.
 14. In about the first week of January 2009, the Appellant's family provided Dr. Cudmore with the results of their research into treatment facilities for the Appellant, and advised that they wished the Appellant to receive treatment at The Canyon in California, as it had immediate openings, no clean time was required prior to arrival and there was a 2:1 patient staff ratio. The facility advised that it could meet the Appellant's needs, provide him with a one-on-one intervention therapist to assist with the brain injury related deficits, in addition to treating the addictions.
 15. Dr. Cudmore submitted a Prior Approval Application for Full Payment of Insured Out-of-Country Health Services to the Ministry of Health and Long-Term Care on January 5, 2009 for acquired brain injury and cocaine addiction, indicating that treatment was required immediately, and that a delay could result in death or irreversible tissue damage as a possibility from an accidental overdose.
 16. By letter dated January 7, 2009, the Ministry notified the Appellant that it had refused the Prior Approval Application. The letter of refusal indicated that the Drug and Alcohol Registry of Treatment (DART) case reports that the Ministry had obtained through ConnexOntario showed that there were "multiple facilities throughout Ontario that would consider treating clients with brain injuries," and thus that treatment was available in Ontario.
 17. On that same date, the Appellant was admitted to the program at The Canyon in California where he received residential services from January 7 to March 23,

2009. The Appellant appeals the Respondent's decision to deny payment for the services rendered at The Canyon.

III. LAW

Insured Services under the *Health Insurance Act*

18. Health insurance in Ontario is governed by the provisions of the *Health Insurance Act* (the *Act*) and regulations made pursuant to the *Act*.
19. Section 12 of the *Act* provides that every insured person is entitled to payment for insured services.
20. "Insured services" is defined in sections 1 and 11.2 of the *Act* to include only those services that are prescribed by regulation.
21. For the most part, insured services under the *Act* are health services rendered in Ontario. Section 28.4 of Regulation 552 (the Regulation) made under the *Act* also provides a scheme for payment for out-of-country health services when the services are rendered outside of Canada with prior approval of OHIP because they are not performed in Ontario, or there would be a delay in Ontario that would result in death or medically irreversible tissue damage.
22. The provisions of Regulation 552 pertaining to out-of-country medical services were amended effective April 1, 2009. Because the medical services at issue in this appeal were received prior to April 1, 2009, the provisions of Regulation 552 as they were prior to April 1, 2009 are applicable to this appeal. A portion of section 28.4 of the pre-April 1, 2009 Regulation reads as follows:

28.4(2) Services that are part of a treatment and that are rendered outside Canada at a hospital or health facility are prescribed as insured services if,

- (a) the treatment is generally accepted in Ontario as appropriate for a person in the same medical circumstances as the insured person; and
- (b) either,
 - (i) that kind of treatment that is not performed in Ontario by an identical or equivalent procedure, or
 - (ii) that kind of treatment is performed in Ontario but it is necessary that the insured person travel out of Canada to avoid a delay that would result in death or medically irreversible tissue damage.

23. Section 28.4 (5) sets out the conditions of payment for out-of-country insured services as follows:

28.4 (5) The following are conditions of payment of amounts for services prescribed in this section:

- 1. An application for approval of payment must be submitted to the General Manager by a physician who practices medicine in Ontario on behalf of the insured person and the application must contain a written confirmation from that physician that, in the opinion of the physician, one of the conditions set out in clause 2 (2) (b) is satisfied.

IV. ISSUES

24. The Respondent confirmed that the medical services that the Appellant received at Canyon are generally accepted as appropriate treatment for a person in the Appellant's medical circumstances; therefore, the appropriateness of treatment is not in dispute.
25. The Appeal Board must determine if the treatment the Appellant received at Canyon was performed in Ontario by an identical or equivalent procedure, and if it was performed in Ontario, whether it was necessary for the Appellant to travel out of Canada to avoid a delay that would result in death or irreversible tissue damage.

V. ANALYSIS

26. The onus is on the Appellant to demonstrate, on a balance of probabilities, that treatment was not performed in Ontario by an identical or equivalent procedure, and if it was, that it was necessary for him to travel out of Canada to avoid a delay that would result in death or irreversible tissue damage.
27. The Appeal Board will review the evidence on these issues. In addition to written submissions and documentation received in advance of the oral hearing, the Appeal Board heard oral testimony during the course of three hearing dates, April 19, May 26 and October 13 2010, from the Appellant's father, Mr. P.T., the Appellant's sister, Ms. R.T., Dr. Robert Thomson, a medical consultant with the Ministry of Health and Long- Term Care adjudicating out of country health service applications, and Dr. Duncan Scott, a psychiatrist called as an expert witness for the Respondent.
28. The Appellant filed medical records describing his medical condition following the motor vehicle accident. In a 2006 Neuropsychiatric Assessment report, which had included two days of testing, Dr. R. Van Reekum confirmed that the Appellant suffered a severe TBI and that under Ontario legislation, the TBI would have been considered catastrophic. According to this 2006 report, the prognosis was for little if any further spontaneous recovery. Dr. Van Reekum noted that aggressive and ongoing treatment would allow for the optimization of quality of life, wellness and functional abilities.
29. Dr. Cudmore filed the Prior Approval Application on behalf of the Appellant. His clinical notes were not entered as an exhibit to the hearing. At the hearing, Appellant's Counsel declined to enter them on the basis that they did not contain anything of substance. Dr. Cudmore indicated to the Respondent's staff that he had been the Appellant's physician for two years, and that the Appellant had not been receiving treatment for his addiction.

30. Dr. C. Mathew, a neurologist, is noted on the Prior Approval Application to be the only other Ontario physician with whom the Appellant consulted regarding the condition for which treatment was being sought out of country. The Appellant filed a letter from Dr. C. Mathew dated December 11, 2007.
31. In December 2007, Dr. Mathew assessed the Appellant. The Appellant's father testified that the purpose of the assessment was for reinstatement of the Appellant's driver's license, and that the consultation was 20 minutes long.
32. Dr. Mathew does not state why the Appellant was referred to her, although at the close of the letter, she concludes that the Appellant has made very good recovery considering the severity of his head injury, that he has residual mild cognitive impairment, but that it will not impact his ability to drive safely, and finally states that the Appellant should have a practical driving assessment prior to reinstatement of license.
33. Dr. Mathew reviews the Appellant's clinical history in the letter, makes note of his social and personal history and the findings of her physical examination, and indicates that she conducted the MOCA (the Montreal Cognitive Assessment) test, in which he scored 24/30, with normal being over 26/30.
34. The Appellant also filed the discharge summary, but no clinical notes, from his treatment at Canyon. The Canyon discharge summary identified the Appellant's Axis I diagnosis as "mood disorder due to traumatic brain injury, with mixed features, mixed receptive-expressive language disorder, attention deficit hyperactivity disorder, NOS, relational problem related to general medical condition, cocaine dependence, cannabis dependence and alcohol abuse". His Axis III diagnosis was traumatic brain injury, aggressive type and Axis IV diagnosis was problems with primary support group, problems related to the social environment, occupational problems, and other psychosocial and

- environmental problems. The Canyon discharge report indicated that the Appellant had made sufficient progress in his treatment to continue treatment at a lower level of care, and made a number of recommendations following discharge, including abstinence, individual and couple therapy, speech therapy and engagement in a 12-step program.
35. Finally, the Appellant filed a psychological report dated February 5, 2009, prepared by Yellen & Associates in Northridge, California. The author of the report, Andrew Yellen, Ph.D., concluded, “much of [the Appellant’s] malfunctioning may be attributed to his TBI. However, since his language difficulties were identified by premorbid assessments, his ability to properly and effectively mediate his emotions is also a contributing factor”. He set out a series of recommendations including ongoing therapy including for his language disorder, monitoring for substance abuse, follow-up by a neurologist, work on social skills and a re-evaluation of the Appellant’s pharmacological regimen.
36. As noted above, in the spring of 2008, the Appellant’s family contacted Homewood to inquire about the possibility of residential treatment at the facility for the Appellant. At the oral hearing, the Appellant’s father testified that a staff member at Homewood advised him that the facility would be unable to treat the Appellant due to the information provided about his brain injury. When the family contacted Homewood in May 2008, they did not mention the brain injury, and were advised that an application needed to come through a referral from the patient’s family physician. Dr. Cudmore then made a referral to Homewood. The document from Dr. Cudmore, referring the Appellant to Homewood, was not included in the evidence filed at the hearing.
37. The Appellant’s father testified that he believed that his son would have thrown out any mail from Homewood at the time. He acknowledged that his son was not prepared to enter rehabilitation until late 2008, but that the family was “trying to get our ducks in order” so that when he was ready to go, they could take him. The

- Appellant's father testified that he and his wife understood that there was a 6 – 12 month waiting period for entry to Homewood.
38. According to the Appellant's written submissions, on December 29, 2008, following the incident with the baseball bat, the Appellant's mother made the following inquiries:
- a. She contacted the DART and spoke with an individual called Nigel, who informed her that he did not know of any Rehab Centres that dealt with both TBI and addictions. He referred her to the Centre for Addiction and Mental Health (CAMH).
 - b. Upon contacting CAMH, the Appellant's mother was referred back to an individual at St. Michael's hospital who was researching TBI. Some days later, that individual from St. Michael's returned the call and referred the family back to CAMH.
 - c. The Appellant's mother contacted Roger Palmanteer at Bellwood Health Services. He advised that he would speak with the team. He did so and called back to state that the team at Bellwood would take the Appellant and he wanted to set up a telephone assessment with the Appellant. He was to call back with the date and time of the assessment. Mr. Palmanteer then called back and said that the Director of Bellwood advised they could not treat TBI and substance abuse/addiction.
39. After receiving the Appellant's funding application, the Ministry obtained from ConnexOntario, a DART case report listings of 13 residential treatment programs in Ontario for substance abuse identifying acquired brain injury as matching requirement. The programs were in Timmins, London, North Bay, Ottawa, Thunder Bay, Scarborough (Bellwood Health Services Inc), Toronto (Salvation Army & CAMH), Guelph, Port Colborne, Glencairn, and Kingston. A few had estimated availability some time in January 2009. Some listings included notes requiring individual assessment or specifying capability of program participation. Homewood was not included in the DART case report printouts obtained by the Ministry.

40. The Appellant's written submission to the Board included an undated list of 21 facilities in the United States and Canada, including CAMH in Ontario, contacted by the Appellant's sister-in-law, T.T., with regard to the Appellant. The Appellant's sister and father stated that initially all of the treatment centres contacted indicated that they could accept the Appellant into their programs, but that once the Appellant's family gave particulars about the severity of the Appellant's acquired brain injury, they indicated that he was not a suitable candidate as the treatment programs would not be able to meet the demands of his brain injury given their staff to client ratios.
41. The Appellant's sister testified at the hearing. She stated that she works as a catastrophic rehabilitation consultant specializing in brain injury. She testified that on March 6, 2009, a few weeks before the Appellant returned from The Canyon, she contacted some of the programs listed on the Ministry's DART report, including Bellwood, which indicated that it did not treat individuals with brain injuries together with other substance addictions.
42. The Appellant's father testified that when contacting facilities in Ontario on behalf of his son, he would describe the Appellant's condition as he saw it. He testified:
- They asked us if he was agitated; yes. Would he be able to sit in a crowded room with 10 to 15 people? We said no. Does he need one-on-one? In some cases, yes. And, from what I saw at The Canyon, he needed a lot more than I even anticipated that he needed.
- There was lots of times that C. – when there were five people in the room at The Canyon, that C. would leave the room and get very frustrated, and he would have to leave, and there was somebody there to calm him down on a regular basis.
43. At the hearing, when the Appellant's sister was asked how she described the Appellant's combination of his brain injury and addiction issues when she spoke with facilities about treatment, she responded that she did this as well as she was able, and had given clear examples of things that recently had occurred with the

- Appellant. The goal was to find a place that could accept him. The main reason why she was told that a facility was not appropriate was the ratio of workers to clients – stating typically it was one worker to every 10 to 15 people and that the Appellant required a lower ratio because of his acquired brain injury.
44. Finally, the Appellant's sister testified at the hearing as follows: the family sought to go through the channels available to them; had her brother been accepted at Homewood, he would have gone there at the end of 2008; Homewood would have been a preferred option as the family could have travelled to Guelph to support him; and her father accompanied her brother some 3000 kilometres to The Canyon for the three-month period.
45. The Respondent submitted two written opinions from Dr. Duncan Scott in connection with this case, dated March 16, 2009 and May 21, 2010, and called him as a witness at the hearing. Dr. Scott is a psychiatrist who has specialized in alcohol and drug rehabilitation and comorbid emotional and psychiatric problems.
46. Dr. Scott did not examine the Appellant directly but had medical documentation regarding the Appellant – in particular, the discharge summary from The Canyon Centre, dated March 23, 2009; a psychologist's report of Dr. Yellin, dated February 5, 2009; and Dr. Mathews' December 2007 report. He did not have Dr. Cudmore's clinical records.
47. Dr. Scott, in his report to the Respondent of March 16, 2009, stated that he reviewed Dr. Mathew's letter of December 11, 2007 and that in his opinion, the Appellant was capable of going through an addiction program, that there was no need for any concurrent program regarding his acquired brain injury, and that there are Ontario centres that address issues of addiction and co-morbidities related to his traumatic brain injury:

There are internationally acclaimed centres such as Bellwood and Homewood that address issues of addiction and co-morbidity with regard to depressive and obsessive features, anxiety related features and any of the features that this patient may be demonstrating that may be residual from his traumatic brain injury. I see no reason why this individual cannot attend any of the addiction units in Ontario.

48. In his oral testimony, Dr. Scott testified that patients who need treatment for addictions issues very often have other co-morbidities including brain injury, for which they require treatment and that any of Ontario's addiction treatment centres could treat a patient such as the Appellant who had suffered a traumatic brain injury. Dr. Scott defined co-morbid diagnosis as meaning that a patient has one problem such as addiction, together with another psychiatric illness. He stated that recovery addiction units deal with co-morbidity routinely, elaborating in his oral testimony as follows:

Addiction is a result of many, many issues, a rejection of loss and neglect. It can also have to do with prior traumas, and it's an individual's way of trying to cope with the outside stressors, and they reach to these chemical comforts to provide relief. So, generally, there are many other issues with regard to the individual that comes in for treatment for their substance use dependence....[Homewood has] many different areas to help with comorbidities of those with addiction in treatment centres and post-traumatic difficulties, particularly depression....There's about four diagnoses in Axis I, that any of the addiction treatment centres in Ontario could handle. Certainly could.

49. Dr. Scott submitted that the Appellant's situation was no different from that of many individuals who have harmed themselves cognitively with frontal damage through overdoses and chronic use of cocaine and other substances. He stated that the majority of individuals coming into an addiction treatment facility have mild to moderate brain injury because of drug usage for example. Dr. Scott opined that the Appellant's impairment level was such that he did not need an inpatient residential centre where the *primary diagnosis* was of traumatic brain injury with

- co-resultant alcohol and drug difficulties and that he could be treated at a facility that did not specialize in brain injury. He stated that upon review of the treatment process the Appellant underwent at The Canyon, no one with catastrophic injuries could have participated in the intensive inpatient therapy the Appellant engaged in, including psycho-education and alcoholics anonymous steps.
50. Specifically, regarding the report from Yellin and Associates and The Canyon discharge summary, Dr. Scott stated that the contents of those reports affirm his view that treatment could have been provided in Ontario. He opined further that based on the reports that were available to him, it appeared that the Appellant had experienced dramatic cognitive improvement between 2004 and 2007. In his opinion, the Appellant's impairment level did not require an inpatient treatment facility for traumatic brain injury, and that based on what he saw in the discharge summary from The Canyon, the treatment that the Appellant had at The Canyon was available to him in Ontario.
 51. The Appellant's Counsel questioned Dr. Scott as to why in his report he had not referred to the findings of Dr. van Reekum, particularly that the Appellant would experience little, if any, spontaneous recovery. Dr. Scott stated that Dr. van Reekum seemed to be incorrect. Although he was not questioned further about this, the Appeal Board infers that Dr. Scott was comparing Dr. van Reekum's view with the findings of Dr. Mathew.
 52. Upon questioning by the Appeal Board, Dr. Scott stated that Dr. Mathew's opinion as to the Appellant's level of cognitive impairment should not be diminished because she apparently was assessing the Appellant for the purposes of reinstatement of a driver's license, in light of the risks and responsibilities inherent in operating a vehicle. In other words, she would have taken the assessment seriously given the serious responsibility inherent in operating an automobile. On the other hand, he stated that the fact that the Appellant displayed

only mild cognitive impairment in December 2007 is not inconsistent with the behaviours he displayed in November and December 2008, explaining as follows:

...from my understanding of most injuries, or most people who suffer traumatic injuries such as this one, and the limitations, and remarkable improvement, the body usually improves much quicker than the mind does. He was taken out of his social circles. He certainly had difficulties in relating to his peers. But, even prior to that, this was – kind of, exacerbated. When you look at Dr. van Reekum's report in early childhood, he was having problems early on in school, also, with a diagnosis of Attention Deficit Disorder that wasn't treated for well over 10 years.

So, we have a pre-disposing factor with an individual, then we have a traumatic incident, then we have some physical problems which kind of repair on their own, but then consequently, we have cognitive problems that are going on that leave him in a rather distressed state, and he's reaching for chemical comforts of which he had used in the past, sometimes alcohol when he was young, to help cope with the traumatic changes in his life....

53. Regarding the family's difficulty in accessing treatment as described by the Appellant's father and sister, Dr. Scott testified that the system relies on physician referral, and that with physician involvement, suitable and appropriate placement in a treatment facility could have been found. Dr. Scott stated that in this case, the Appellant's family was not well placed to identify the Appellant's needs to prospective medical facilities from a medical perspective, and did not seek the active assistance of an Ontario physician to access appropriate treatment.
54. In summary, it was Dr. Scott's opinion that based upon his evaluation of the Appellant's condition as it was in January 2009, appropriate treatment was performed in Ontario at that time, in particular at Homewood or Bellwood, and would have been available to the Appellant if properly accessed.

a) Was identical or equivalent treatment performed in Ontario?

55. The onus is on the Appellant to establish on a balance of probabilities that identical or equivalent treatment was not performed in Ontario. In order to meet that onus, the Appellant must demonstrate with reliable evidence that identical or equivalent treatment was not performed in Ontario at the material time. The Appeal Board must weigh the evidence provided by both parties to determine if the Appellant has met this onus.
56. The statutory scheme for payment of out-of-country medical services demands that an Ontario medical practitioner provide an opinion as to whether treatment identical or equivalent to that which was received out of country, was performed in Ontario. The requirement for this opinion is set forth in s. 28.4(5) of Regulation 552, which provides that an application for prior approval be filed in which a physician who practices medicine in Ontario gives the opinion that one of the conditions in s. 28.4 (2) (2) (b) is met.
57. In this case, the Appellant relies upon the conclusions formed by the Appellant's family members based upon their contact with Homewood and other facilities, and the form the Appellant's family physician, Dr. Cudmore, filled out at their request. The information provided by Dr. Cudmore consisted of him ticking off the box on Part 4A of the application for prior approval and giving the answer "No" to the question, "Is this treatment performed in Ontario by an identical or equivalent procedure?" Dr. Cudmore does not elaborate upon the information he provided in the form. He did not testify at the hearing. His clinical records were not entered as an exhibit to the hearing. No information was advanced to explain Dr. Cudmore's experience in addictions medicine. No other Ontario medical practitioner testified or provided a written report on behalf of the Appellant.
58. The Respondent countered with the opinion of Dr. Scott. It was Dr. Scott's view upon reviewing the Appellant's clinical records that identical or equivalent

- treatment was and is performed in Ontario at addiction treatment facilities including Homewood and Bellwood.
59. Upon a careful consideration and weighing of this evidence, the Appeal Board finds that the Appellant has failed to meet the onus of establishing on a balance of probabilities that identical or equivalent was not performed in Ontario, and the appeal must fail on that basis.
60. The Appellant's father testified that the Appellant's family first contacted Homewood in the spring of 2008 and that based on the information they provided, Homewood advised that it would not be a suitable placement for the Appellant. The evidence is that in May 2008, the Appellant's family contacted Homewood again and did not mention that the Appellant had an acquired brain injury. Homewood advised that the facility required a referral from a family doctor.
61. Subsequently, through Dr. Cudmore, an application was made to Homewood. The actual application was not included in the evidence filed at the hearing. Apparently, Homewood then mailed material to the Appellant; however, Homewood advised Dr. Cudmore, by letter dated July 24, 2008, that the Appellant's name had been removed from the waiting list because it had not received the requested information from the patient necessary to offer admission. The Appellant's father testified that in all likelihood his son would have thrown out any information received from Homewood. This is not surprising because, as noted below, it was not until December 27, 2008 that the Appellant acknowledged that he required treatment.
62. There is no evidence that the Appellant or his family asked Dr. Cudmore to pursue the Homewood referral and there is no evidence that Dr. Cudmore made any further inquiries of Homewood.

63. The Appellant's father contacted Homewood and asked if he could provide the requested information as he held the Appellant's Power of Attorney. Homewood advised that would not be acceptable. Neither the family nor the Appellant made any further contact with Homewood.
64. It was not until December 27, 2008, following the incident with the baseball bat, that the Appellant agreed that he needed further treatment, and was prepared to enter a course of treatment.
65. At no time after December 27, 2008 did the Appellant or his family pursue a referral to Homewood or have any further contact with that facility, nor is there any evidence that they sought the assistance of Dr. Cudmore.
66. The Appellant's sister testified that the Appellant's mother contacted CAMH, Bellwood and St. Michael's Hospital and in each instance, upon describing the Appellant's condition, was told suitable treatment was not available. This evidence, together with the Prior Approval form upon which Dr. Cudmore ticked the box indicated treatment was not performed in Ontario without delay, and the family's prior dealings with Homewood, is the foundation of the Appellant's argument that identical or equivalent treatment was not performed in Ontario. There is no evidence that Dr. Cudmore made any inquiries himself regarding the availability of treatment in Ontario and so it is a reasonable assumption that in ticking off the form, he relied on the information provided to him by the Appellant's family.
67. Dr. Scott, who had provided two written expert opinions, filed by the Respondent, testified at the hearing that placement at a treatment facility in Ontario is done via physician referral, which involves the patient's physician providing the relevant medical information to the prospective facility. Dr. Scott's testimony was that this physician to physician referral system ensures that the appropriate information is transmitted to the prospective facility so that an accurate assessment can be made

- as to the suitability of that facility given the patient's medical condition, as described by a medical practitioner and not by a family member.
68. Dr. Scott testified that in his opinion, having reviewed the Appellant's medical records, identical or equivalent treatment was performed in Ontario at Homewood and Bellwood, and the Appellant failed to access that treatment in the appropriate manner, via physician referral. He testified that all the therapies the Appellant received at Canyon are available in Ontario, with the exception of equine assisted psychotherapy and adventure experimental therapy, submitting that there is no evidence-based medicine to support the use of those therapies. There is no evidence that those particular therapies were critical to the course of treatment the Appellant required.
69. The Appellant sought to discredit Dr. Scott's opinion on the basis that he relied more heavily on the conclusions of Dr. Mathew than those of Dr. van Reekum. Dr. Scott opined that Dr. van Reekum seemed to be incorrect as to the Appellant's prospects for recovery, and he explained how Dr. Mathew's assessment as at December 2007 was not inconsistent with the Appellant's subsequent behavior in November and December 2008. In addition, the Appeal Board notes that Dr. Mathew's assessment was more recent than that of Dr. van Reekum. Furthermore, on the application for funding, Dr. Mathew appears as the only other Ontario physician with whom the Appellant had consulted regarding his condition of "acquired brain injury" and "cocaine addiction", so Dr. Scott was relying on the medical conclusions of a treating physician of the Appellant, and the only one listed on the funding application other than Dr. Cudmore himself.
70. The Appellant relies upon the communications with Homewood in the spring and summer of 2008 to submit that treatment was not available to him at Homewood in late December 2008/early January 2009.

71. The Appeal Board is not persuaded by the Appellant's submission in this regard. The Appellant was not prepared to admit that he needed treatment until December 27, 2008. No attempt to contact Homewood was made at that time, either directly by the Appellant, his family, or through Dr. Cudmore. There is no evidence that Dr. Cudmore or any other Ontario physician saw the Appellant prior to his departure for The Canyon, for the purpose of assessing his treatment needs at that time. Based on the totality of the evidence on the appeal, the Appeal Board concludes that Dr. Cudmore did not play an active role in assessing the Appellant's treatment requirements.
72. The Appeal Board is troubled by the written submission filed on behalf of the Appellant that the Appellant's mother contacted Bellwood by telephone in late December 2008 and after initially advising that the facility could offer treatment, in a subsequent phone call, retracted that and indicated that it could not offer treatment for traumatic brain injury and addictions. The Appellant's father testified that after many requests, he finally received a letter from Bellwood confirming that Bellwood would not take the Appellant. This letter was not included in evidence at the hearing. Given the lack of physician involvement in the communications with Bellwood, the absence of direct information as to Bellwood's understanding of the Appellant's needs, and Dr. Scott's opinion that the required treatment was performed at Bellwood, the Appeal Board is not prepared to conclude that Bellwood could not treat the Appellant.
73. Payment for out-of-country medical services is part of a statutory scheme that requires the opinion of an Ontario physician that one of the conditions in s. 28.4 (2) is met. That opinion must be reliable. In this case, the Appeal Board is not persuaded by the ticking off of the appropriate box on the application for prior approval by Dr. Cudmore that identical or equivalent treatment was not performed in Ontario. In the context of this appeal, the Appeal Board finds that the opinion of Dr. Scott as a specialist engaged in addictions medicine in Ontario, that

- identical or equivalent treatment was performed in Ontario at Homewood and Bellwood, is more reliable than that of Dr. Cudmore.
74. The Appeal Board appreciates that the Appellant's family felt that they needed to act quickly once the Appellant conceded that he required treatment. It is often the case that the families of insured persons become involved in researching treatment options, both within and outside of Ontario, and patients and their families might self-refer to an out-of-country medical facility. However, in order for that service to be insured under the statutory scheme, the Appellant must meet the requirements of Regulation 552.
 75. The Appellant referred that The Canyon is an OHIP out-of- country preferred provider and the Respondent confirmed this fact. However, the fact that a particular facility is listed as a preferred provider does not mean that services received by an insured person will be insured by OHIP. The insured person is still required to satisfy the requirements of the *Health Insurance Act* and regulations in order for the services to be considered insured services.
 76. The Appeal Board finds that the Appellant has failed to establish on a balance of probabilities that identical or equivalent treatment was not performed in Ontario.

(b) Was it necessary for the Appellant to travel outside of Canada to avoid a delay in obtaining treatment in Ontario that would result in death or medically significant irreversible tissue damage?

77. In order to demonstrate that it was necessary for the Appellant to travel outside of Canada to avoid a delay in Ontario that would result in death or medically significant irreversible tissue damage, the Appellant must demonstrate on a balance of probabilities that: (i) there would have been a delay in obtaining treatment in Ontario; and (ii) that delay would have resulted in death or medically

- significant irreversible tissue damage. Section 28.4(5) requires that an Ontario physician confirm this information in the Application for Prior Approval.
78. The Appeal Board finds that the Appellant has failed to discharge the onus of proof on either of these issues.
79. The Appeal Board has found that identical or equivalent treatment was performed at Homewood at the material time, and that the Appellant did not attempt to access treatment at Homewood in late December 2008/early January 2009. The Appellant's family indicated that when they made enquiries with Homewood earlier in 2008, they understood that there was a waiting period for Homewood's treatment program. However, there is no specific evidence before the Appeal Board as to what the delay, if any, would have been at Homewood at the time that the Appellant sought treatment at The Canyon.
80. On the Application for Prior Approval, Dr. Cudmore ticked off the boxes in Part 4 indicating that there would have been a delay in receiving treatment in Ontario, during which time tissue damage was "possible if has accidental OD [overdose]" and that treatment was required "immediately". Dr. Cudmore ticked off the box indicating that identical or equivalent treatment was not performed in Ontario. Perhaps he meant that if he was incorrect and treatment was performed in Ontario, there would be a delay that possibly could result in an accidental overdose. Dr. Cudmore does not identify the length of the delay.
81. Even if the Appellant had established on a balance of probabilities that there was a delay in receiving treatment in Ontario, the Appellant has not demonstrated that he required treatment within a particular period in order to avoid death or medically significant irreversible tissue damage.
82. Dr. Cudmore's information on the Application for Prior Approval is the only direct medical evidence the Appellant has filed on the issue of the consequences

of delay in treatment. As indicated in the above paragraph, that information is scant. The preponderance of evidence establishes that Dr. Cudmore had very little, if any, involvement in researching or considering treatment options for the Appellant, or in attempting to access treatment for the Appellant beyond the referral to Homewood earlier in 2008. Dr. Cudmore's records were not filed, nor were any hospital records filed from the Appellant's treatment following the baseball bat incident. No other medical evidence was filed to suggest that the Appellant was at risk of overdose.

83. There is no information in the Discharge Summary from The Canyon to suggest that there was a time frame within which the Appellant required treatment in order to avoid the risk of death or medically significant irreversible tissue damage, due to overdose or any other cause.
84. The Appellant's family was understandably very concerned about the Appellant's safety. However, the Appeal Board is not persuaded on the evidentiary record before it that the Appellant has established on a balance of probabilities that there would have been a delay in receiving treatment in Ontario, or that the delay would have resulted in death or medically significant irreversible tissue damage.

VI. DECISION

85. The Appellant has failed to demonstrate that treatment identical or equivalent to what he received at The Canyon was not performed in Ontario, nor has he demonstrated that it was necessary to travel out of Canada to avoid a delay in obtaining treatment in Ontario that would have resulted in death or medically significant irreversible tissue damage. As such, the treatment the Appellant received at The Canyon is not an insured service.

86. The appeal is denied.

ISSUED January 30th, 2012

Soraya Farha

Taivi Lobu

Elizabeth Mullan